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Sleep, Recovery & Mental Resilience

Sleep, recovery and stress: a free quick start

FREE QUICK START

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Contents

Start here

Why sleep is the foundation

Set up the night

Caffeine, alcohol, food and supplements

The evening routine

When sleep does not improve

Your 14-day plan

Disclaimer. This educational resource presents sleep, recovery and stress-management strategies for personal development. It does not constitute medical, psychological or therapeutic advice. The author is a nutrition coach, not a licensed healthcare provider. If you have persistent insomnia, symptoms of sleep apnea, a mental health condition, ADHD, a substance dependence or any other clinical concern, consult a qualified professional before making changes. Cold exposure and breathing practices are not suitable for everyone. Individual results vary.

FREE QUICK START

Start here

This is the free quick start of Sleep, Recovery & Mental Resilience. It covers why sleep matters, setting up the night, caffeine and alcohol, an evening routine, when to see a doctor and a 14 day plan. The complete guide adds ADHD and sleep, recovery, resilience and worksheets.

The complete guide

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CHAPTER 1

Why sleep is the foundation

How much do adults need?

A consensus statement from the American Academy of Sleep Medicine and the Sleep Research Society recommends that adults sleep **7 or more hours per night** on a regular basis to promote optimal health. Sleeping less than 7 hours regularly is associated with adverse health outcomes, including weight gain, diabetes, high blood pressure, heart disease and impaired performance.[1] Individual needs vary a little. A useful test: if you need an alarm to wake up, nap easily in the day or rely on caffeine to function, you may be short.

Sleep and appetite

In a laboratory study of 12 healthy young men, two nights of 4-hour sleep reduced leptin (a hormone that signals fullness) by about 18% and increased ghrelin (a hunger hormone) by about 28%, and the men reported about a 24% increase in hunger and stronger appetite for calorie-dense foods.[2] The sample was small and the exposure extreme, but the direction matches what many people feel after a bad night: more hunger, more cravings.

Sleep and fat loss

In a small randomized crossover study, 10 overweight adults followed the same calorie-restricted diet for two weeks, once with 8.5 hours in bed and once with 5.5 hours. On both occasions they lost a similar amount of **weight,** but on the short-sleep arm they lost about **55% less fat** and about 60% more lean mass.[3] It was a small study, so do not treat the exact percentages as precise. The practical point stands: if you diet while short of sleep, you may lose a worse kind of weight.

Sleep and performance

In a study of Stanford basketball players, extending sleep to about 10 hours per night for five to seven weeks was associated with faster sprint times, better shooting accuracy and improved mood.[4] There was no control group, so some of this may be due to other factors. A review of sleep in athletes concluded more broadly that sleep loss impairs performance and recovery and that sleep extension is a promising strategy.[5]

CHAPTER 2

Set up the night

1. A consistent schedule

Go to bed and wake up at about the same time every day, within an hour or so, including weekends. This is a standard recommendation in sleep medicine, because a regular schedule stabilizes your body clock. If your schedule is irregular because of shifts or kids, the principle still holds: anchor **one** thing, usually the wake-up time.

2. Light: bright by day, dim by night

Light is the strongest signal for your body clock. A practical rule that matches the science of circadian rhythms:

- **Get outdoor light soon after waking,** ideally 10 to 20 minutes. Even a cloudy sky is far brighter than indoor lighting.
- **Dim the lights in the evening.** Lamps instead of bright overhead lights for the last 1 to 2 hours.
- **Dark room at night.** Blackout curtains or a sleep mask. Cover small LEDs.

In a controlled study, 12 adults who read on a light-emitting e-reader for several evenings before bed took longer to fall asleep, felt less sleepy at night, had **suppressed melatonin** and a later body clock, and were less alert the next morning compared with reading a printed book.[6] A device on a low brightness setting is a smaller problem than a bright one, and what you do on it matters too (a stressful email is worse than a book), so the sensible rule is: **screens off or very dim for the last hour,** and the phone charges outside the bedroom.

3. Cool, quiet, comfortable

Core body temperature falls as you go to sleep, and a room that is too warm makes that harder. A review of thermal environment and sleep found that heat exposure tends to increase wakefulness and reduce deep sleep and REM.[7] Many sleep guides suggest a bedroom around 16 to 19 degrees C (60 to 67 F). Treat that range as a starting point and adjust to what feels right for you.

Room setting	Aim	Cheap fix
Temperature	Cool, a little cooler than you think	Fan, lighter bedding, window open, lower thermostat
Light	As dark as possible	Blackout curtains, sleep mask, tape over LEDs
Noise	Quiet or steady	Earplugs, white noise or fan
Bed	Used for sleep and sex only	No work, no scrolling in bed

CHAPTER 3

Caffeine, alcohol, food and supplements

Caffeine: later than you think

Caffeine has a half-life of roughly 5 to 6 hours in many adults, so half of an afternoon dose is still active at bedtime. In a randomized crossover trial, 400 mg of caffeine taken 0, 3 or 6 hours before bedtime significantly reduced total sleep time, **even when taken 6 hours before bed** (objectively about 40 minutes less sleep), and people did not always notice it.[8] A sensible rule: **stop caffeine at least 8 hours before bed,** and earlier if you are sensitive. For many people that means no coffee after about 1 to 2 PM.

You may have read that you should delay your first coffee for 90 to 120 minutes after waking so that adenosine clears. This is a popular idea, but I could not find human trials showing that it improves energy or sleep. If waiting suits you, fine, but it is not required. Do not feel guilty about a morning coffee.

Alcohol: a sedative, not a sleep aid

Alcohol may help you fall asleep faster, but a systematic review found that it reduces REM sleep in the first half of the night and disrupts sleep in the second half, with more waking and lighter sleep.[9] Higher doses have larger effects. If you drink, finish at least 3 to 4 hours before bed, drink water alongside, and keep it modest. If you are working on cutting alcohol out, expect your sleep to feel worse for a few weeks before it improves, which is common, and talk to your doctor if you were drinking heavily.

Food and timing

- **Avoid a very large meal right before bed.** It can cause discomfort and reflux.
- **Do not go to bed hungry** either. A small protein-rich snack is fine (Greek yogurt, a glass of milk).
- **Spread fluids across the day** so you are not waking for the toilet.

Melatonin: modest and specific

A meta-analysis of 19 studies in people with sleep problems found that melatonin reduced the time to fall asleep by about 7 minutes and increased total sleep by about 8 minutes, a small effect.[10] It can be helpful for shifting your body clock (jet lag, shift work, or a delayed schedule), generally in low doses taken at the right time. Supplement products vary in actual dose. It is not a substitute for fixing the habits above, and you should check with your doctor if you take other medication, are pregnant or have a medical condition.

A note on other sleep supplements

Magnesium, glycine, theanine, ashwagandha and others are popular. Evidence for most is limited or mixed. Habits first. If you want to try one, add it alone, for two weeks, and keep notes.

CHAPTER 4

The evening routine

Wind down on purpose

Your brain does not switch off like a lamp. Give it a runway of 30 to 60 minutes. The activity matters less than the pattern: the same cues, in the same order, signal "sleep is coming."

1. **60 minutes before bed:** screens off or very dim. Dim the lights. Do the tomorrow list on paper.
2. **45 minutes before:** a warm shower or bath, if you like. Brush teeth.
3. **30 minutes before:** something calm. A book, stretching, slow breathing, quiet music.
4. **At bedtime:** lights out, room cool and dark. If you cannot sleep within about 20 minutes, see the tip below.

A warm shower helps (a bit)

A meta-analysis found that a warm bath or shower (about 40 to 42.5 degrees C) taken **1 to 2 hours before bed** improved sleep quality and shortened the time to fall asleep by about 10 minutes.[11] The proposed reason is that the body cools after the heat, which helps trigger sleep. A very hot shower right before bed may do the opposite for some people.

Exercise helps sleep

A meta-analysis found that both single sessions and regular exercise improved sleep, with small to moderate effects, including less time to fall asleep and more deep sleep.[12] For most people, exercise at any time is better than none. If you find evening workouts hard to wind down from, move the hard sessions earlier and keep the evening gentle.

Racing mind? Write it down

If your mind races at bedtime, do a two-minute "brain dump": list what you are worried about and one next step for each. Then put the paper away. This is a coaching habit with limited direct evidence, but it costs two minutes.

If you cannot sleep

If you are still awake after about 20 minutes, get up and do something quiet in dim light (read a book, stretch) and go back when sleepy. Do not look at the clock. Staying in bed, frustrated, teaches your brain that bed means struggle.

CHAPTER 5

When sleep does not improve

Insomnia is treatable, and it is not just "bad habits"

If you have trouble falling or staying asleep at least three nights a week for three months or more, and it affects your day, you may have insomnia disorder. The first-line treatment is **cognitive behavioral therapy for insomnia (CBT-I),** not sleeping pills. The American College of Physicians recommends CBT-I as the initial treatment for chronic insomnia in adults.[13] A meta-analysis of 20 randomized trials found that CBT-I improved insomnia severity, sleep onset latency and total sleep time in adults, with durable effects.[14]

CBT-I component	What it is
Sleep restriction	Temporarily limit time in bed to your actual sleep time, then widen it. Do this with guidance.
Stimulus control	Bed only for sleep. Get up if awake more than about 20 minutes.
Cognitive work	Challenge unhelpful sleep beliefs ("I will be useless tomorrow").
Relaxation and routine	Wind-down habits, breathing, a fixed schedule.

CBT-I is delivered by therapists, by programs and by apps, and is widely available online. Ask your doctor.

Signs to see a doctor

- Loud snoring, gasping, or pauses in breathing at night, or morning headaches. These can mean **sleep apnea,** which is common, serious and treatable.
- Falling asleep at the wheel or in dangerous situations.
- Restless legs or kicking at night.

- Insomnia that lasts weeks despite good habits, or that comes with low mood or severe anxiety.
- You rely on alcohol or over-the-counter pills to sleep every night.

A caution about sleeping pills

Prescription and over-the-counter sleeping pills may help in the short term but carry risks and are not first-line treatment for long-term insomnia. Do not start or stop prescribed sleep medication without your doctor.

CHAPTER 6

Your 14-day plan

Days	Change	Why this order
1 to 3	Fix your **wake-up time** and get 10 minutes of outdoor light	Anchors your body clock. Costs nothing.
4 to 6	**Caffeine cutoff** 8 hours before bed. Phone charges outside the bedroom	Biggest sleep gains for most people.
7 to 9	**Wind-down routine:** dim lights, screens off for the last hour, brain dump	Makes sleep come easier.
10 to 12	**Room:** cool, dark, quiet. Alcohol off on weeknights	Improves sleep quality.
13 to 14	Review: sleep log, mood, energy. Decide what stays	You keep what works.

Keep a simple log for the two weeks: bedtime, wake time, how you slept (1 to 5), energy at 3 PM (1 to 5). Do not obsess over numbers, since worrying about sleep can make it worse. The pattern over two weeks is what counts.

Want more? The complete guide has every chapter, worksheets and the full reference list. adrianonutrition.com/guides

Questions?

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